NCCP 00376 · VERSION 4

Exemestane · daily hormone therapy

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Exemestane: material risks

☐ Joint and muscle pain or stiffness

☐ Vaginal dryness and sexual symptoms

☐ Lipid / cardiovascular risk changes

☐ Hot flushes, sleep disturbance and fatigue

☐ Bone thinning and fractures

☐ Uncommon liver abnormalities or serious allergic reactions

NCCP does not require a routine renal or hepatic dose reduction. This does not remove the need to assess organ dysfunction, interactions and tolerability. Confirm the postmenopausal endocrine state. Any ovarian-suppression combination needs a separate prescription and monitoring plan.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile, confirmation of menopause when uncertain, and bone / fracture assessment. Consider baseline vitamin D as specified by the product information. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. Review symptoms, adherence and bone health. Repeat bone density and lipids when indicated; assess other blood tests according to the clinical plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending